



Intended-Use / Indications Statement

ARKA Health, Inc. — Pre-Submission · Document 03 · Version 1.0 · June 9, 2026

A. ARKA-CLIN — intended use

ARKA-CLIN is intended for use by licensed health care professionals to support the selection of clinically appropriate diagnostic imaging for adult and pediatric patients in elective and non-urgent care settings. Using structured clinical data, ARKA-CLIN presents ranked, evidence-cited imaging options drawn from published clinical guidelines and peer-reviewed literature, together with the basis for each option, so that the clinician can independently review each recommendation and retains full responsibility for the decision. ARKA-CLIN does not acquire, process, or analyze medical images or physiological signals; does not provide time-critical alerts or triage; and does not place, cancel, or block orders. It is intended to support, not replace, the clinician's independent judgment.

Criterion alignment

Wording	Alignment sought
"licensed health care professionals" / "support, not replace"	Criterion 3 (recommendations to an HCP, not a directive) and Criterion 4 (independent judgment retained)
"structured clinical data ... does not acquire, process, or analyze medical images or physiological signals"	Criterion 1 (no image/signal acquisition, processing, or analysis)
"published clinical guidelines and peer-reviewed literature ... basis for each option"	Criterion 2 (well-understood and accepted sources) and Criterion 4 (reviewable basis)
"ranked ... options" (plural)	Multiple options presented, supporting the statutory-exclusion analysis rather than the enforcement-discretion lane described in the January 2026 guidance for single-output CDS
"elective and non-urgent ... no time-critical alerts or triage"	Addresses the time-critical considerations discussed under Criterion 4 in the January 2026 guidance

B. ARKA-INS — function statement (administrative, §520(o)(1)(A))

ARKA-INS is intended for use by health care providers and qualified staff to determine health-benefit eligibility, analyze historical claims data to estimate future utilization and



cost, support prior-authorization documentation, and present cost-transparency and scheduling information. These are administrative-support functions of a health care facility under §520(o)(1)(A) and are not intended to provide clinical diagnosis or treatment recommendations.

C. Intended users

Licensed physicians (MD/DO), nurse practitioners, physician assistants, radiologists, and qualified staff acting under clinician supervision. Not intended for patient or caregiver use.

D. Evidence sources

ARKA-CLIN recommendations are anchored to published clinical practice guidelines and peer-reviewed literature from recognized bodies and professional societies, including the ACR (American College of Radiology) Appropriateness Criteria and related ACR publications, USPSTF recommendations, Choosing Wisely campaign statements, and specialty-society guidelines (e.g., AAP, ACEP, AAOS, ACOG, ACP/AAFP, NCCN). Every shipped card carries a structured citation to its source, maintained in a canonical citation registry.

E. Explicitly out of scope

- Medical image or signal analysis of any kind (CT, MRI, X-ray, ultrasound, ECG, CGM, SpO₂, and similar)
- Time-critical alerts or triage (e.g., stroke, sepsis, STEMI pathways)
- Autonomous order placement, cancellation, or blocking
- Patient- or consumer-facing recommendations
- The reference DICOM viewer (a separate display function not under review)

ARKA requests FDA's feedback on whether this intended use, as implemented in the design described in Document 02, supports a Non-Device CDS determination for ARKA-CLIN under §520(o)(1)(E).